MERIDIAN BAY MEDICAL CENTER

PHARMACY & THERAPEUTICS COMMITTEE

INSTITUTIONAL FORMULARY DIGEST

Selected Monographs — Adult Inpatient Use

FICTIONAL DOCUMENT — FOR SOFTWARE TESTING ONLY. All drug names, doses, indications, contraindications, and clinical guidance in this document are entirely fictional. Nothing in this document represents real pharmacology and it must not be used for any clinical purpose.

Preface

This digest is maintained by the Meridian Bay Medical Center Pharmacy & Therapeutics Committee and represents the institutional reference for adult inpatient prescribing of the medications listed herein. Each monograph reflects the most current evidence and the committee's consensus position on safe prescribing parameters for our patient population. The digest is reviewed quarterly. The most recent revision was approved on February 12, 2026.

Prescribers should consult this digest before initiating, modifying, or counseling on any of the listed medications. Where institutional guidance differs from manufacturer labeling, the institutional guidance reflects committee consensus on appropriateness for our patient mix and should be followed. Cases requiring deviation from the digest should be discussed with the on-call pharmacist or with the Pharmacy & Therapeutics Committee directly.

Scope of this digest: Adult inpatient and discharge prescribing only. Pediatric, obstetric, geriatric, and outpatient-specialty prescribing parameters may differ and are addressed in separate institutional documents. The absence of a medication from this digest does not necessarily mean the medication is not on formulary — only commonly prescribed and high-volume agents are included here. Prescribers requiring guidance on agents not listed in this digest should consult the inpatient pharmacy directly.

I. Cardiovascular Agents

This section covers antihypertensives, rate-control agents, and renin-angiotensin system modulators used in the management of hypertension, atrial fibrillation, and heart failure in the adult inpatient population.

Velantine

Drug class: Angiotensin-converting enzyme (ACE) inhibitor.

Indication: First-line antihypertensive in adult patients without contraindication. Also used in selected patients with heart failure with reduced ejection fraction and in post-myocardial-infarction prophylaxis.

Standard adult dose: 10 mg orally once daily. Initial dose may be reduced to 5 mg in patients with mild renal impairment or in patients at risk of first-dose hypotension. Titrate upward at intervals of no less than two weeks based on blood pressure response and tolerability.

Maximum dose: 40 mg orally once daily.

Contraindications: Pregnancy (all trimesters; teratogenic). History of angioedema with any ACE inhibitor. Bilateral renal artery stenosis. Concurrent use with an angiotensin receptor blocker.

Significant interactions: Concurrent use with NSAIDs increases risk of acute kidney injury, particularly in volume-depleted patients. Concurrent use with potassium-sparing diuretics or potassium supplements increases risk of hyperkalemia. Concurrent use with angiotensin receptor blockers is contraindicated due to lack of additional benefit and substantially increased risk of renal impairment, hypotension, and hyperkalemia.

Monitoring: Serum creatinine and potassium within 1–2 weeks of initiation and after any dose change. Blood pressure at each clinic visit.

Common adverse effects: Dry cough (10–15% of patients), dizziness, hyperkalemia. Rare but serious: angioedema (any time during therapy, not only at initiation).

Prescribing notes: If a persistent dry cough develops, consider switching to an angiotensin receptor blocker rather than dose reduction.

Cordizem-XR

Drug class: Non-dihydropyridine calcium channel blocker, extended release.

Indication: Rate control in atrial fibrillation and atrial flutter. Also used as second-line antihypertensive in patients with concurrent need for rate control. Not first-line for hypertension alone.

Standard adult dose: 50 mg orally once daily, taken in the morning. May be titrated upward in increments of 25 mg at intervals of no less than two weeks based on heart rate response and blood pressure. Sustained-release formulation; must not be crushed or chewed.

Maximum dose: 200 mg orally once daily.

Contraindications: Severe bradycardia (heart rate < 50 bpm). Second- or third-degree atrioventricular block in the absence of a functioning pacemaker. Severe left ventricular systolic dysfunction. Concurrent use with intravenous beta-blockade.

Significant interactions: Concurrent use with beta-blockers increases risk of bradycardia and AV block; combination should be undertaken only with explicit cardiology input. Cordizem-XR is a moderate CYP3A4 inhibitor; doses of statins metabolized by CYP3A4 may need adjustment.

Monitoring: ECG within 1–2 weeks of initiation and after any dose change. Resting heart rate and blood pressure at each clinic visit.

Common adverse effects: Bradycardia, hypotension, fatigue (particularly in the first 1–2 weeks), peripheral edema.

Prescribing notes: In diabetic patients, Cordizem-XR may blunt the adrenergic warning signs of hypoglycemia. Patients with diabetes should be counseled to monitor blood glucose more frequently in the first month of therapy.

Mirosartan

Drug class: Angiotensin II receptor blocker (ARB).

Indication: Antihypertensive, particularly in patients who cannot tolerate ACE inhibitors (e.g., ACE-inhibitor-induced cough). Also used in selected patients with heart failure with reduced ejection fraction.

Standard adult dose: 80 mg orally once daily. Initial dose may be reduced to 40 mg in elderly patients or in patients at risk of first-dose hypotension. Titrate upward at intervals of no less than two weeks based on blood pressure response.

Maximum dose: 160 mg orally once daily.

Contraindications: Pregnancy (all trimesters; teratogenic). Bilateral renal artery stenosis. Concurrent use with an ACE inhibitor (contraindicated combination).

Significant interactions: Concurrent use with ACE inhibitors is contraindicated. Concurrent use with NSAIDs increases risk of acute kidney injury. Concurrent use with potassium-sparing diuretics increases risk of hyperkalemia.

Monitoring: Serum creatinine and potassium within 1–2 weeks of initiation and after any dose change.

Common adverse effects: Dizziness, hyperkalemia, fatigue. Angioedema is rare but reported.

Prescribing notes: Mirosartan does not cause the dry cough characteristic of ACE inhibitors and is the preferred renin-angiotensin system agent for patients with prior ACE-inhibitor intolerance.

II. Lipid-Lowering Agents

Pravoxil

Drug class: HMG-CoA reductase inhibitor (statin).

Indication: Primary and secondary prevention of atherosclerotic cardiovascular disease in patients with elevated LDL cholesterol or established cardiovascular disease.

Standard adult dose: 20 mg orally once daily, at bedtime. Pravoxil's pharmacokinetics favor evening administration to align peak inhibition with the nighttime peak of endogenous cholesterol synthesis. Morning administration substantially reduces therapeutic effect.

Maximum dose: 80 mg orally once daily, at bedtime. Doses above 40 mg should be reserved for patients who have not achieved LDL targets on lower doses and are tolerating the medication well.

Contraindications: Active liver disease or unexplained persistent elevation of serum transaminases. Pregnancy and breastfeeding. Concurrent use with strong CYP3A4 inhibitors at doses above 20 mg.

Significant interactions: Grapefruit juice substantially increases Pravoxil plasma concentrations and should be avoided. Concurrent use with strong CYP3A4 inhibitors (e.g., certain antifungals, certain protease inhibitors) increases risk of myopathy and rhabdomyolysis.

Monitoring: Baseline liver function tests (LFTs), repeat at 6–12 weeks after initiation. CK only if symptoms of myopathy develop.

Common adverse effects: Myalgia (5–10% of patients), elevated LFTs, gastrointestinal upset. Rare but serious: rhabdomyolysis, hepatotoxicity.

Prescribing notes: Patients should be counseled to take Pravoxil at bedtime, not in the morning. Patients should report any unexplained muscle pain, tenderness, or weakness, particularly when accompanied by malaise or dark urine.

III. Anxiolytics and Sedatives

Etrazolam

Drug class: Benzodiazepine, intermediate-acting.

Indication: Short-term management of acute anxiety, situational anxiety, and selected cases of insomnia secondary to anxiety. Not appropriate for long-term anxiety management; SSRIs or other non-benzodiazepine agents should be used for chronic anxiety.

Standard adult dose: 0.5 mg orally twice daily. Initial dose may be reduced to 0.25 mg twice daily in elderly patients (age ≥ 65) or patients with hepatic impairment.

Maximum dose: 1 mg orally twice daily, with maximum daily dose of 2 mg.

Contraindications: Known hypersensitivity to benzodiazepines. Severe respiratory insufficiency. Acute narrow-angle glaucoma. Myasthenia gravis. History of substance use disorder is a relative contraindication and requires specialty consultation.

Significant interactions: Concurrent use with opioids substantially increases risk of respiratory depression and is generally to be avoided. Alcohol, other CNS depressants, and certain antihistamines potentiate sedation.

Monitoring: Reassessment of clinical need at every visit. Patients should be reassessed for continued need at 2 weeks and again at 4 weeks.

Common adverse effects: Sedation, dizziness, ataxia, anterograde amnesia. Risk of falls in elderly. Risk of dependence with prolonged use.

Prescribing notes: Etrazolam is approved for short-term use only, defined as a treatment duration of up to 4 weeks including any taper period. Use beyond 4 weeks substantially increases risk of physical dependence and is not supported by efficacy evidence for this indication. Prescribers must not initiate Etrazolam with an intended duration beyond 4 weeks. If anxiety persists beyond 4 weeks, the patient should be transitioned to a non-benzodiazepine agent under appropriate specialty supervision. Discontinuation should be by gradual taper, not abrupt cessation, to avoid withdrawal symptoms.

IV. Anticoagulants

Sumavast

Drug class: Direct oral factor Xa inhibitor.

Indication: Stroke prevention in non-valvular atrial fibrillation. Treatment and secondary prevention of deep vein thrombosis and pulmonary embolism.

Standard adult dose: 5 mg orally twice daily. Dose may be reduced to 2.5 mg twice daily in patients meeting any two of the following criteria: age ≥ 80, body weight ≤ 60 kg, or serum creatinine ≥ 1.5 mg/dL.

Maximum dose: 5 mg orally twice daily. Higher doses do not provide additional anticoagulant benefit and substantially increase bleeding risk.

Contraindications: Active clinically significant bleeding. Hepatic disease associated with coagulopathy. Concurrent use with other anticoagulants except during careful transitions.

Significant interactions: Concurrent use with strong dual CYP3A4 and P-glycoprotein inhibitors substantially increases Sumavast exposure and bleeding risk; combination should be avoided. NSAIDs, antiplatelet agents, and SSRIs all incrementally increase bleeding risk.

Monitoring: Renal function at baseline and at least annually. Hepatic function at baseline. Periodic assessment of bleeding history and concomitant medications.

Common adverse effects: Bleeding (minor and major), nausea, anemia. Rare but serious: intracranial hemorrhage, gastrointestinal hemorrhage.

Prescribing notes: Sumavast does not require routine INR monitoring. Patients should be counseled to report any unusual bleeding or bruising, black or tarry stools, or unexplained anemia symptoms. In the event of major bleeding or emergency surgery, a specific reversal agent is available; the on-call hematology service should be contacted.

V. Gastrointestinal Agents

Quintaxin

Drug class: Proton pump inhibitor.

Indication: Treatment of gastroesophageal reflux disease, peptic ulcer disease, and prophylaxis against NSAID-induced gastropathy in high-risk patients. Also used as prophylaxis in patients on multi-drug regimens with elevated risk of medication-induced gastritis.

Standard adult dose: 20 mg orally once daily, taken 30 minutes before breakfast. Morning administration before the first meal of the day produces the most reliable acid suppression.

Maximum dose: 40 mg orally once daily for refractory cases or as initial therapy for severe esophagitis.

Contraindications: Known hypersensitivity to proton pump inhibitors. Concurrent use with rilpivirine or atazanavir.

Significant interactions: Reduces absorption of medications requiring gastric acidity (ketoconazole, certain iron formulations). May reduce the activated metabolite of clopidogrel; clinical significance is debated.

Monitoring: Reassessment of continued indication at 8 weeks. Long-term users should have periodic assessment of magnesium and B12 levels.

Common adverse effects: Headache, diarrhea, abdominal discomfort. Long-term use associated with B12 deficiency, hypomagnesemia, increased fracture risk (particularly hip and vertebral), and increased risk of Clostridioides difficile infection.

Prescribing notes: Quintaxin should not be used long-term without periodic reassessment. Patients prescribed Quintaxin for prophylaxis should have the indication reviewed at 8 weeks; continuation beyond this point requires affirmative clinical justification.

VI. Antihistamines

Floranase

Drug class: Second-generation H1-receptor antagonist.

Indication: Symptomatic relief of seasonal and perennial allergic rhinitis, chronic urticaria, and selected cases of mild allergic conjunctivitis.

Standard adult dose: 10 mg orally once daily.

Maximum dose: 10 mg orally once daily. There is no additional benefit at higher doses.

Contraindications: Known hypersensitivity to Floranase. Severe hepatic impairment.

Significant interactions: No significant CYP-mediated interactions. Concurrent use with alcohol may potentiate any sedation, although Floranase is classified as non-sedating.

Monitoring: None routinely required.

Common adverse effects: Headache, dry mouth, mild fatigue. Sedation is uncommon at standard doses but can occur, particularly in elderly patients.

Prescribing notes: Floranase is preferred over first-generation antihistamines for daytime use due to its substantially lower sedation profile. Patients should still be counseled that mild sedation is possible and should be cautious when first starting therapy if planning to drive or operate machinery.

VII. Quick-Reference Summary

Practitioners should reference the full monograph above before prescribing. The following summary is provided for quick lookup only and is not a substitute for the detailed entries.

Standard adult doses

Velantine 10 mg PO daily. Cordizem-XR 50 mg PO daily. Mirosartan 80 mg PO daily. Pravoxil 20 mg PO at bedtime. Etrazolam 0.5 mg PO twice daily, short-term only. Sumavast 5 mg PO twice daily. Quintaxin 20 mg PO daily, 30 minutes before breakfast. Floranase 10 mg PO daily.

Critical prescribing constraints

- Pravoxil must be administered at bedtime, not in the morning.
- Etrazolam treatment duration must not exceed 4 weeks.
- Mirosartan must not be combined with any ACE inhibitor, including Velantine.
- Quintaxin should be reassessed for continued need at 8 weeks.
- Cordizem-XR maximum daily dose is 200 mg; standard dose is 50 mg daily.

Drugs not in this digest

This digest contains the institutional monographs for the eight medications above. Other medications may be on formulary at Meridian Bay Medical Center but are not addressed in this digest. Prescribers requiring guidance on a medication not listed here should contact the inpatient pharmacy directly. Do not infer prescribing parameters for unlisted medications from the entries above.

VIII. Document Control

Document owner: Pharmacy & Therapeutics Committee, Meridian Bay Medical Center.

Revision: 8.2 (February 12, 2026).

Next scheduled review: May 12, 2026.

Distribution: All credentialed inpatient prescribers and inpatient pharmacy staff.

Reminder: *This is a fictional document created for software-testing purposes. It does not represent real pharmacological guidance and must not be used for any clinical purpose.*